

VIDEO 16 · PRODUCTION PACKET

The Real Reason Your Bloating Won't Go Away

A shoot-ready packet: script, thumbnail, clips, and a shoot-day checklist

Prepared for Alina Nazari, FDN-P · Solo, 17 to 19 minutes

GO HAPPY BELLY

ROOT-CAUSE GUT HEALTH

Alina, start here. Section 1 is your script, written the way you would say it. Read it through twice before filming. You do not need to memorize it, just get familiar. The three lines marked as key moments are the ones worth saying word for word. This one carries more validation weight than usual, she has likely been dismissed more than once, so let the warmth sit before the direct lines land. Everything after the script (thumbnail, clips, checklist) is for Steve and the editor, so you can skip it unless you are curious.

WHAT'S INSIDE

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SECTION 1

The Script

Length: 17 to 19 min · **Format:** solo to camera · **One idea:** recurring bloating is not about a food you have not found yet, it is bacteria living in the wrong part of your gut, and the 14-day reset addresses that, not another list of foods to cut.

Delivery: conversational, warm, direct. This one carries more validation weight than usual, talk to the woman who has already done the "right" things and is still bloated. The small side-notes and colored labels are for editing, do not read them aloud. The 14-day reset described here is educational, something Alina teaches, not a personal challenge she is running on camera, so this is shoot-ready now.

0:00 – 0:35 Hook

You have done the elimination diet. You cut gluten, then dairy, then half the vegetables in your kitchen. You took the probiotic. For a week or two it got better. And then, without changing anything, the bloating came back. Same as before, sometimes worse.

Here is the real reason that happens, and it is not that you missed a trigger food. In the next few minutes I am going to show you why bloating relapses even when you did everything right, the specific reason it keeps returning that almost nobody explains to you, and the simple two-week reset that actually addresses it instead of just managing around it. Stay with me, because the reason it comes back is not what you think, and once you see it you will stop blaming yourself for "falling off" a diet that was never going to fix this in the first place.

ON SCREEN: WHY IT KEEPS COMING BACK

EDITOR Open the "why it relapses" loop now, pay it off at Point 2. B-roll: a fridge with a "safe foods" list, a hand closing a food journal, a woman pressing on her stomach at her desk.

0:35 – 1:15 Credential moment

Quick hello, because you should know who is telling you this. I am Alina Nazari, I am a Functional Diagnostic Nutrition Practitioner, and most of the women I work with have already been to two, three, sometimes five different doctors about this exact bloating. The

labs came back normal. Someone told them it was stress. Someone told them to eat smaller meals. And they left feeling like either their body is broken or they are imagining how bad it actually feels.

You are not broken, and you are not imagining it. Your labs just were not looking at the right thing. Let me show you what they were missing.

VISUAL Warm, direct to camera. Lower third with name and FDN-P appears here.

1:15 – 3:00 **The relapse cycle**

Let me describe the cycle, and tell me if this is you. You get bloated, so you assume it is something you ate. You start cutting. First the obvious things, then the less obvious things, until your "safe list" is smaller than what you actually eat in a normal day. You feel a little better for a while. Then, with no clear trigger, it comes right back, and now you are more restricted than before and still bloated.

And here is what that does to a person over time. You start doubting your own body. You start wondering if you are just sensitive to everything, or worse, if you are the problem. Meanwhile every test your doctor ran comes back "normal," which somehow makes it feel even more confusing, because normal labs are supposed to mean you are fine, and you very much do not feel fine.

ON SCREEN: NORMAL LABS. STILL BLOATED.

Here is what I want you to understand before we go any further. The fact that your labs are normal and you still feel this way does not mean nothing is wrong. It means the standard panel was never designed to look at the thing that is actually happening. Let me show you what that thing is.

3:00 – 6:00 **Point 1 • It's not a food, it's a neighborhood**

Your gut is supposed to work like a house with very clear rooms. Your large intestine is where most of your bacteria are meant to live, in large numbers, doing useful work. Your

small intestine, right before it, is supposed to stay relatively quiet, mostly bacteria-free, because that is where you actually absorb your nutrients.

For a lot of women dealing with bloating that will not quit, some of those bacteria have relocated. Not new bacteria, not bad bacteria, the same bacteria that are supposed to be there, just living in the wrong room. When they end up in the small intestine in bigger numbers than they should, they start fermenting food earlier than they are supposed to, right where you are trying to absorb it. That fermentation makes gas. The gas makes you bloat, often within twenty, thirty minutes of eating, sometimes so much you look pregnant by the end of the day.

ON SCREEN: WRONG ROOM, NOT WRONG FOOD

This is what I call the Bloated-and-Backed-Up pattern, bacteria that moved to the wrong neighborhood. And here is the line I want you to take with you, because it explains why cutting foods never fully worked.

"The bacteria aren't the problem. The location is."

KEY LINE Deliver this slowly, it reframes the whole video. B-roll: simple non-clinical animation of a house floor plan, a room labeled correctly versus a room with figures that do not belong there, moving in.

6:00 – 9:00 Point 2 • Why it always comes back, the flooded basement

Now here is the part that answers the question I opened with, why the bloating comes back even after you did the diet right.

Cutting certain foods can genuinely calm things down for a while, because you are giving those relocated bacteria less to ferment. But think about what you are actually doing. You are managing the water on the floor. You are not fixing the leak that keeps letting the water back in.

Picture a basement that keeps flooding. You can mop it up every single time, and the floor looks dry for a few days. But if there is an actual leak somewhere, a crack, a pipe, something letting water back in, the basement floods again. Mopping never was going to

fix a leak. Cutting foods is the mop. It never touches the reason bacteria keep drifting back into the wrong room in the first place.

ON SCREEN: YOU'VE BEEN MOPPING THE FLOOR

That is the relapse you have felt. You did not fail the diet. The diet was never designed to reach the leak. This is the recurrence problem, and it is the single biggest reason women tell me they have "tried everything" and nothing sticks.

PAYOFF *This closes the loop from the hook. B-roll: a basement floor with a little water, someone mopping, then the water slowly seeping back in from a small crack. Keep it clearly metaphorical, not a medical illustration.*

9:00 – 11:15

Point 3 • Why the standard labs missed it

I promised I would show you why the normal labs did not catch this, so let me actually explain it, because you deserve a real answer, not "your labs are fine, must be stress."

A standard blood panel looks at things like your complete blood count, your basic metabolic markers, maybe your thyroid. Useful information, but none of it is designed to ask "where exactly are your bacteria living, and is fermentation happening earlier in your digestive tract than it should." That is a completely different question, and it requires a completely different kind of test to even ask it.

So when your doctor says your labs are normal, they are telling you the truth about the labs they ran. They are just not telling you those labs were never aimed at this pattern in the first place. Normal results and a real, physical pattern are not contradictions. They are just two different maps of two different territories.

ON SCREEN: NORMAL RESULTS. WRONG MAP.

VISUAL *Split screen, a generic lab report on one side, a simple digestive tract diagram on the other, to separate "the map they checked" from "the territory that matters here."*

11:15 – 13:45

Point 4 • The signs this pattern is yours

So how do you know if this is actually what is going on with you? Here are the signs I look for.

One. The bloating shows up fast, often within thirty to sixty minutes of eating, and it gets visibly worse as the day goes on, flatter in the morning, distended by evening.

Two. Certain healthy foods make it worse, not better, especially beans, onions, garlic, certain fruits, the exact foods that get labeled "gut healthy" everywhere online. That is often a clue you are dealing with fermentation happening in the wrong place, not a food that is bad for you in general.

Three. It came on or got dramatically worse after a round of food poisoning, a course of antibiotics, or a stretch of high stress, any of the things that can disrupt the normal cleanup rhythm of your gut.

Four. You have tried an elimination diet, felt better briefly, and relapsed with no clear new trigger, exactly the pattern we already walked through.

And five, the one almost nobody connects. You often feel worse on probiotics, not better, at least at first. If adding "good bacteria" made your bloating spike, that is not you being broken, that is a strong clue this is a location issue, not a not-enough-good-bacteria issue.

ON SCREEN: PROBIOTICS MADE IT WORSE? THAT'S A CLUE.

EDITOR *On-screen checklist builds one line at a time as she names each sign.*

13:45 – 16:45

Point 5 • The 14-day reset that actually addresses it

So if the mop was never going to fix the leak, what does?

This is where I want to introduce something I teach my clients, a simple 14-day reset built to give your system's natural cleanup rhythm a real chance to work again, instead of just avoiding foods forever. I want to be clear about what this is. It is not a permanent restriction. It is a short, structured window, and here is what it focuses on.

First, spacing. You give real space between meals, four to five hours, instead of grazing all day, because your gut has a natural cleaning wave between meals that sweeps leftover

material through, and that wave barely gets a chance to run if you are never actually done eating.

Second, a temporary lower-fermentation way of eating for those fourteen days specifically, not because those foods are bad, but because giving the wrong-room fermentation a break for two weeks is part of how the reset works. This is short-term and strategic, not a forever list.

Third, supporting your body's own digestive rhythm, things like eating without rushing, chewing fully, and giving your body cues that it is safe to run its normal maintenance in between meals.

Fourteen days is not random. It is enough time to interrupt the cycle and let your system's own cleanup process catch up, without turning into another permanent, exhausting list of foods you can never eat again. Most women tell me the bloating noticeably eases somewhere in that second week, which makes sense, because you are not managing symptoms anymore, you are letting the actual system reset.

And I want to say this clearly. Sometimes fourteen days is not enough on its own, and that is not a failure either. If the pattern keeps returning after a reset like this, that is useful information, and it is worth looking at more closely with someone who can actually see where the recurrence is coming from, rather than trying yet another diet by yourself.

VISUAL Warm, practical. Show a simple plated meal with visible spacing cues (a clock in frame), keep it hopeful, not clinical.

16:45 – 17:30 Bridge / Call to action

If this finally explained why the bloating kept coming back no matter what you cut, do one thing for me, subscribe, because I make videos like this every week for women who have been told their labs are normal and left to figure the rest out on their own.

And if you are sitting there thinking, okay, this actually sounds like me, which pattern is mine, I built a short quiz that walks you through it. It is free, it is in the description, no pressure. It will point you to which gut pattern fits what you are feeling.

GROW-FIRST *Subscribe is the main ask. Quiz is a soft mention only, link in the description and pinned comment. No call or paid offer in this video.*

17:30 – 18:00 **Outro**

And if you noticed the bloating tracks your stress as much as your plate, worse on hard weeks, quieter on vacation, there is a whole other piece to this that has nothing to do with what you are eating and everything to do with your nervous system. I made a video on exactly that. I will put it right here, go watch that one next.

You did not fail the diet, and you are not the problem. The location was. Now you know what to actually fix.

ON SCREEN: THE LOCATION WAS THE PROBLEM

HEADS UP FOR STEVE This outro points to the stress-and-bloating video (YT-005), which may still be packaged but not filmed or live when this video goes up, since both are first-batch P1s. Confirm YT-005 is filmed and live before publishing this video so the end-screen link resolves. If it is not live yet, soften the outro to a plain subscribe ask and add the end-screen link once YT-005 is up. See the checklist.

SECTION 2

Thumbnail Brief

For the designer. Modeled on the proven pattern (face + strong expression + a 2 to 4 word overlay + high contrast), rebuilt in the Go Happy Belly palette.

THE ONE GLANCE

This is not another "cut this food" video. Something else keeps bringing the bloating back, and this video finally names it. The viewer should feel "wait, that's why it keeps coming back" and click to find out what "it" is.

TEXT ON THE THUMBNAIL

- **Primary (4 words max):** IT'S NOT YOUR FOOD
- **Secondary (smaller, terracotta):** KEEPS COMING BACK

VISUAL CONCEPT

- **Subject:** close crop of Alina, shoulders up, direct eye contact, a knowing, steady, slightly conspiratorial expression, the look of someone about to tell you something nobody else has told you. Not shock, not alarm, resolve.
- **Background & color:** solid navy (#1B2D4F) so face and text pop. Primary text in warm cream/white with a subtle dark outline. Secondary text in terracotta (#C67B5C). A thin sage (#7BA987) accent used sparingly.
- **Visual device (pick one):** a small circular "repeat" or cycle-arrow icon (two curved arrows chasing each other), clean and bold, placed low and small, to cue "this keeps happening" without any organ or medical graphic. (Alt: a simple house outline with a small dot moving from one room to another, echoing the "wrong neighborhood" idea.)

LAYOUT

- Face on the right ~45%, text stacked on the left third, device low and small.
- Test at phone-thumbnail size first. If the words are not readable on a phone at a glance, cut a word (drop to "NOT YOUR FOOD").
- Keep the same face framing, palette, and text style as YT-002 and YT-005 so the channel is recognizable in a grid.

COMPLIANCE No condition named, no "SIBO," no "cure" or "treat." On-brand palette, not Ben's yellow and red. Next step: generate 2 to 3 concept mockups, pick a direction, then a designer finalizes with Alina's real photo. A/B idea for later: "IT'S NOT YOUR FOOD" versus "WHY IT KEEPS COMING BACK" (trimmed to fit).

SECTION 3

Repurposing Plan

For the editor. Five clips cut from the long-form for Reels, TikTok, and YouTube Shorts. Pull the highest-retention moments.

#	Timestamp	Angle	Caption	Hook on screen
1	3:00–4:45	Your bloating might not be a food problem at all. "The bacteria aren't the problem. The location is."	Your bloating might not be about what you're eating. It might be about where things are happening. Full video on the channel.	IT'S NOT YOUR FOOD
2	6:00–7:30	Why cutting foods calms bloating for a while, then it comes right back.	If your bloating always comes back no matter what you cut, you're mopping a floor with a leak nobody fixed. Here's what that means.	YOU'VE BEEN MOPPING THE FLOOR
3	9:00–10:30	Why normal bloodwork does not catch this pattern, and what that means if you've been told you're fine.	Normal labs and still bloated? Your labs were never looking at this.	NORMAL LABS. WRONG MAP.
4	11:15–13:00	Five signs the recurring bloating is a location pattern, not a food you haven't found yet.	5 signs your bloating keeps coming back for a reason, not because you keep "cheating."	5 SIGNS IT'S NOT THE FOOD
5	13:45–15:15	What the 14-day reset actually focuses on, and why 14 days is not random.	Want to know what actually breaks the bloating cycle? Not another food list. Here's the real reset.	THE REAL 14-DAY RESET

DISTRIBUTION

- **Order & spacing:** publish the long-form first, then one clip every 2 to 3 days over two weeks. Suggested order: Clip 1, Clip 2, Clip 4, Clip 3, Clip 5.
- **Cross-platform:** same clip to Reels, TikTok, and Shorts. Lead with Instagram (the warm 8K audience).
- **Clip 1** (the "it's not your food" reframe plus the aha line) is the strongest, post it first.
- **CTA (grow-first):** "Full video on the channel, subscribe." Pinned comment: soft quiz mention, "free quiz in bio." No paid offer yet.

- Burn in captions (most watch on mute) and keep the first 2 seconds to the hook line above.

SECTION 4

Shoot Day Checklist & Upload Copy

For Steve. Everything to film it and upload it.

1. BEFORE THE SHOOT

- ☐ Alina reads the script out loud twice. Familiarity, not memorization.
- ☐ Lock the 3 word-for-word lines: the hook, the aha line ("The bacteria aren't the problem. The location is."), and the close ("You did not fail the diet, and you are not the problem. The location was.").
- ☐ Wardrobe: solid warm tones (cream, sage, terracotta, navy). No busy patterns, no pure white or black, no big logos.
- ☐ Location: quiet room, soft daylight or a key light, warm non-clinical background.
- ☐ Water nearby, phone silent, notifications off.

2. GEAR & SETUP

- ☐ Camera at eye level, framed chest-up, eyes on the top third.
- ☐ Light in front and slightly to one side, softened. Avoid overhead-only light.
- ☐ Audio is the priority: lav or shotgun mic close to her, not the built-in mic. Record 10 seconds of room tone.
- ☐ Teleprompter app just under or over the lens so her eyeline stays on camera. Slow scroll.
- ☐ Do a 20-second test: focus, exposure, audio levels.

3. DELIVERY NOTES FOR ALINA

- ☐ Talk to one person, not an audience (someone who already tried the elimination diet and is still bloated).
- ☐ This video carries more validation weight than usual. Let the warmth sit before the direct lines land.
- ☐ Slow down on the hook, the aha line, and the "normal labs, wrong map" line. Pauses read as confidence.
- ☐ Energy slightly higher than feels natural (camera flattens it ~20%).
- ☐ Fine to restart a sentence: pause, clap once (helps the editor), redo the line.
- ☐ The emotional job of this video is taking the self-blame off the table, she did not fail the diet.
- ☐ The 14-day reset here is educational content Alina teaches, not a personal challenge she has to run first. Shoot-ready now, no BTS needed.

4. SHOT LIST

A-roll: film each section as its own take: Hook, Credential, Problem, Points 1 to 5, Bridge/CTA, Outro.

B-roll (10 to 20 sec each, silent):

- ☐ A fridge with a "safe foods" list, a hand closing a food journal (hook/problem).
- ☐ A woman pressing on her stomach at a desk, a lab printout with "normal range" highlighted (problem).
- ☐ A basement floor with a little water on it, someone mopping (Point 2, also a clip).
- ☐ A simple plated meal with a clock visibly in frame, suggesting spacing between meals (Point 5, also a clip).
- ☐ Optional: the "wrong room" floor-plan idea and the leak/basement idea as simple animations added in post, no need to film live.

5. ON-SCREEN TEXT (FOR THE EDITOR)

- 0:00: WHY IT KEEPS COMING BACK | 1:45: NORMAL LABS. STILL BLOATED.
- 3:15: WRONG ROOM, NOT WRONG FOOD | 6:30: YOU'VE BEEN MOPPING THE FLOOR
- 9:15: NORMAL RESULTS. WRONG MAP. | 12:00: PROBIOTICS MADE IT WORSE? THAT'S A CLUE. | 17:45: THE LOCATION WAS THE PROBLEM

6. UPLOAD COPY (PASTE INTO YOUTUBE)

TITLE

The Real Reason Your Bloating Won't Go Away

DESCRIPTION

You did the elimination diet. You cut gluten, then dairy, then half your kitchen. It helped for a while, then the bloating came right back. In this video I walk through the real reason recurring bloating relapses even after you've done everything "right," why your normal labs were never designed to catch this pattern in the first place, the specific signs it fits what I call the Bloating-and-Backed-Up pattern, and the simple 14-day reset that actually addresses it instead of just managing around it.

If this helped, subscribe. I break down root-cause gut health every week for women who were told their labs are normal and left to figure the rest out on their own.

Curious which gut pattern fits what you are feeling? Take the free quiz: <https://gut-pattern-quiz.scoreapp.com>

Chapters:

0:00 Why your bloating keeps coming back
0:35 Who I am
1:15 Normal labs, still bloated
3:00 It's not a food, it's a neighborhood
6:00 Why it always comes back
9:00 Why the standard labs missed it
11:15 5 signs this pattern is yours
13:45 The 14-day reset that actually works
16:45 What to do next

Alina Nazari, FDN-P
Go Happy Belly

PINNED COMMENT

The reframe that changes everything: bloating that keeps coming back after a clean elimination diet is usually not about a food you haven't found yet. It's about bacteria that ended up living in the wrong part of your gut. Cutting foods manages it for a while, but it doesn't fix why it keeps happening. Has your bloating ever come back with no clear new trigger? If you want to know which gut pattern fits what you're feeling, the free quiz is in the description.

TAGS

why am I still bloated, bloating won't go away, bloating keeps coming back, bloated after eating, elimination diet not working, normal labs still bloated, gut health, functional nutrition, digestive bloating causes, bacterial overgrowth bloating, probiotics made bloating worse, FDN-P

OUTRO CAVEAT The outro teases the stress-and-bloating video (YT-005). Both YT-016 and YT-005 are first-batch P1s, so confirm YT-005 is filmed and live before this video's end screen needs to resolve. If it is not live yet, soften the outro to a plain "subscribe so you don't miss it" and add the end-screen link once YT-005 is up.

7. AFTER THE SHOOT

- ☐ Back up the footage in two places before leaving the room.
- ☐ Send footage + this packet to the editor.
- ☐ Generate thumbnail concept mockups, pick a direction, have a designer finalize with Alina's real photo.
- ☐ Do not upload publicly until Steve reviews the edit. Nothing gets posted without Steve's confirmation.

In Gut Health,
Alina Nazari
FDN-P